

SURGICAL PATHOLOGY REPORT

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TCGA-XF-ABHE-01A-PR Redacted



Collected

Ordered by

FINAL DIAGNOSIS:

RADICAL CYSTECTOMY, BILATERAL PELVIC LYMPH NODE DISSECTION, CONTINENT URINARY DIVERSION AND T-POUCH TO URETHRA:

RIGHT URETER (A):

FROZEN SECTION DIAGNOSIS:

- BENIGN

FINAL DIAGNOSIS:

- BENIGN URETERAL TISSUE

- NO DYSPLASIA OR CARCINOMA IDENTIFIED

LEFT URETER (B):

FROZEN SECTION DIAGNOSIS:

- BENIGN

FINAL DIAGNOSIS:

- BENIGN URETERAL TISSUE

- NO DYSPLASIA OR CARCINOMA IDENTIFIED

APICAL URETHRAL MARGIN (C):

FROZEN SECTION DIAGNOSIS:

- BENIGN

FINAL DIAGNOSIS:

- BENIGN URETHRA AND PERIURETHRAL TISSUE

- NO DYSPLASIA OR CARCINOMA IDENTIFIED

BLADDER AND PROSTATE (D):

BLADDER:

- INVASIVE TRANSITIONAL CELL CARCINOMA, GRADE 3-4/4 WITH SQUAMOUS DIFFERENTIATION, EXTENDING THROUGH THE MUSCULARIS PROPRIA AND INTO PERIVESICAL FAT
- RADIAL MARGIN, FREE OF TUMOR
- NO LYMPHOVASCULAR SPACE INVASION IDENTIFIED
- MILD TO MODERATE UROTHELIAL DYSPLASIA AND REACTIVE UROTHELIAL CHANGES
- NO CARCINOMA IN SITU (FLAT LESION) IDENTIFIED

PROSTATE:

- UNILATERAL PROSTATIC ADENOCARCINOMA, GLEASON' S SCORE 6 (3+3), INVOLVING THE LEFT DISTAL PROSTATE
- TUMOR DOES NOT INVOLVE THE CAPSULE OR PERIPROSTATIC SOFT TISSUE
- NO LYMPHOVASCULAR SPACE INVASION IDENTIFIED
- APICAL AND RADIAL MARGINS, FREE OF TUMOR
- BILATERAL SEMINAL VESICLES AND VASA DEFERENTIA, FREE OF TUMOR
- MULTIFOCAL HIGH GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN II-III) IDENTIFIED

RIGHT PARA-CAVAL LYMPH NODES (E):

- NO METASTATIC CARCINOMA IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

RIGHT COMMON ILIAC LYMPH NODES (F):

- NO METASTATIC CARCINOMA IDENTIFIED IN 12 LYMPH NODES EXAMINED (0/12)

LEFT PARA-AORTIC LYMPH NODES (G):

- NO METASTATIC CARCINOMA IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

LEFT COMMON ILIAC LYMPH NODES (H):

- NO METASTATIC CARCINOMA IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

RIGHT LYMPH NODES OF CLOQUET (I):

ICD-O-3
Carcinoma, Transitional cell NOS
8120/3
Site: Bladder NOS
C67.9
JAD 1/23/14

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- NO METASTATIC CARCINOMA IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

RIGHT EXTERNAL ILIAC LYMPH NODES (J):

- NO METASTATIC CARCINOMA IDENTIFIED IN FIVE LYMPH NODES EXAMINED (0/5)

RIGHT OBTURATOR / HYPOGASTRIC LYMPH NODES (K):

- NO METASTATIC CARCINOMA IDENTIFIED IN 14 LYMPH NODES EXAMINED (0/14)

LEFT LYMPH NODE OF CLOQUET (L):

- NO METASTATIC CARCINOMA IDENTIFIED IN TWO LYMPH NODES (0/2)

LEFT EXTERNAL ILIAC LYMPH NODES, LEFT OBTURATOR / HYPOGASTRIC LYMPH NODES (M):

- NO METASTATIC CARCINOMA IDENTIFIED IN 14 LYMPH NODES EXAMINED (0/14)

RIGHT PRESACRAL LYMPH NODES (N):

- NO METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

PRESACRAL LYMPH NODES (O):

- NO METASTATIC CARCINOMA IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

LEFT PRESACRAL LYMPH NODES (P):

- BENIGN FIBROVASCULAR ADIPOSE TISSUE
- NO MALIGNANCY IDENTIFIED

AORTIC BIFURCATION (Q):

- NO METASTATIC CARCINOMA IDENTIFIED IN THREE LYMPH NODES EXAMINED (0/3)

LEFT PROXIMAL URETER (R):

- BENIGN URETERAL TISSUE
- NO DYSPLASIA OR CARCINOMA IDENTIFIED

RIGHT PROXIMAL URETER (S):

- BENIGN URETERAL TISSUE
- NO DYSPLASIA OR CARCINOMA IDENTIFIED

LYMPH NODE SUMMARY: NO METASTATIC CARCINOMA IDENTIFIED IN 77 LYMPH NODES EXAMINED (0/77)

PATHOLOGIC TNM STAGE: BLADDER: pT3bN0MX

PROSTATE: pT2aN0Mx

DIAGNOSIS COMMENT:

A p53 stain is in process and the result of which will be reported in an addendum to follow.

INTRAOPERATIVE DIAGNOSIS:

FROZEN SECTION DIAGNOSIS:

AFS: Right ureter:
- benign

BFS: Left ureter:
- benign

CFS: Apical urethral margin:
- benign

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GROSS DESCRIPTION:

A: The specimen is received fresh from the O.R. and labelled "Right ureter". It consists of a dilated, tubular segment of tan tissue measuring 0.7 cm in length and 0.7 cm in external diameter. At the request of the surgeon, an intraoperative pathologic consultation is performed in order to aid in the care of this patient. The specimen is entirely submitted for frozen in a single cassette.

B: The specimen is received fresh from the O.R. and labelled "Left ureter". It consists of a dilated, tubular segment of tan tissue measuring 0.4 cm in length and 0.4 cm in external diameter. This segment is inked blue. At the request of the surgeon, an intraoperative pathologic consultation is performed in order to aid in the care of this patient. The specimen is entirely submitted for frozen in a single cassette.

C: The specimen is received fresh from the O.R. and labelled "Apical urethral margin". It consists of segments of tan tissue measuring 1.0 x 0.7 x 0.4 cm in aggregate. At the request of the surgeon, an intraoperative pathologic consultation is performed in order to aid in the care of this patient. The specimen is entirely submitted for frozen in a single cassette.

D: The specimen is received fresh from the O.R. and labelled "Bladder & Prostate". It consists of a cystoprostatectomy specimen measuring overall 26.0 x 11.0 x 5.5 cm. Attached is a flap of peritoneal fat measuring 21.0 x 10.0 cm; grossly uninvolved. The bladder measures 8.0 x 8.0 x 6.0 cm and is solid on palpation. The radial margin is inked blue and the specimen is opened along the anterior surface with a Y-shaped incision. Opening of the specimen reveals a soft, friable tan mass measuring 7.0 x 7.0 x 3.0 cm situated along the posterior bladder wall and extending along the right lateral wall. Sections through this tumor reveal extensive mural involvement with septa of white tissue extending into the perivesical fat. The overlying inked radial margin is grossly uninvolved. The right ureter measures 6.5 cm in length with a circumference of 2.0 cm. Exploration of the right ureter reveals involvement by this friable lesion with a grossly ill-defined residual right ureterovesical junction. The left ureter is grossly uninvolved and measures 8.5 cm in length with a circumference of 1.3 cm. The adjacent urothelium shows folds of tan tissue and superficial ulceration, suggestive of prior biopsy sites. Dissection and palpation of the perivesical fat reveal multiple candidate lymph nodes. Also present is a prostate measuring 4.0 x 3.5 x 3.0 cm. The prostatic urethra measures 2.0 cm in length with a circumference of 2.0 cm. Present is a grossly identifiable verumontanum measuring 0.5 x 0.4 x 0.3 cm. The mucosal surface is smooth and glistening and is not excavated. Serial sections through the bladder reveal a mottled yellow-tan to tan interior. No lesions are grossly identified. Representative tissue is submitted in 23 cassettes.

E: The specimen is received in formalin and labelled "Right para-caval lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 5.0 x 3.0 x 2.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. Representative tissue submitted in four cassettes.

F: The specimen is received in formalin and labelled "Right common iliac lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 5.0 x 4.0 x 2.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. Representative tissue submitted in four cassettes.

G: The specimen is received in formalin and labelled "Left para-aortic lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 4.0 x 2.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in two cassettes.

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H: The specimen is received in formalin and labelled "Left common iliac lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 4.0 x 4.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is submitted in three cassettes.

I: The specimen is received in formalin and labelled "Right lymph node of cloquet". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 1.0 x 1.0 x 0.7 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in a single cassette.

J: The specimen is received in formalin and labelled "Right external iliac lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 4.0 x 3.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in three cassettes.

K: The specimen is received in formalin and labelled "Right obturator / hypogastric lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 8.0 x 6.0 x 3.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in six cassettes.

L: The specimen is received in formalin and labelled "Left lymph node of cloquet". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 2.0 x 1.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in a single cassette.

M: The specimen is received in formalin and labelled "Left external iliac lymph nodes, Left obturator / hypogastric lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 4.0 x 4.0 x 2.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. Representative tissue submitted in six cassettes.

N: The specimen is received in formalin "Right pre-sciatic lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 2.0 x 2.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in a single cassette.

O: The specimen is received in formalin and labelled "Pre-sacral". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 5.0 x 2.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in three cassettes.

P: The specimen is received in formalin and labelled "Left pre-sciatic lymph nodes". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 2.0 x 2.0 x 1.0 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in a single cassette.

Q: The specimen is received in formalin and labelled "Aortic bifurcation". It consists of fragments of lobulated yellow-tan fibroadipose tissue measuring 1.0 x 0.5 x 0.3 cm in aggregate. On palpation, multiple candidate lymph nodes are grossly identified. The specimen is entirely submitted in a single cassette.

R: The specimen is received in formalin and labelled "Left proximal ureter". It consists of a tubule segment of tan tissue measuring 1.0 cm in length and 0.6 cm in external diameter. The specimen is entirely submitted in a single cassette.

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S: The specimen is received in formalin and labelled "Right proximal ureter". It consists of a tubule segment of tan tissue measuring 1.3 cm in length and 0.8 cm in external diameter. The specimen is entirely submitted in a single cassette.

SECTIONS:

AFS: frozen section, right ureter all embedded
 BFS: frozen section, left ureter all embedded
 CFS: frozen section, apical urethral margin all embedded
 D1-3: mass
 D4: right ureterovesical junction
 D5: right ureter and proximal margin
 D6: left ureter and proximal margin
 D7: right lateral wall
 D8: central (posterior, dome, anterior)
 D9: bladder neck
 D10: left lateral wall
 D11: right seminal vesical and vas deferens
 D12: left seminal vesical and vas deferens
 D13: candidate lymph nodes
 D14: prostate, right distal
 D15: prostate, right posterior mid
 D16: prostate, right anterior mid
 D17: prostate, right posterior proximal
 D18: prostate, right anterior proximal
 D19: prostate, left distal
 D20: prostate, left posterior mid
 D21: prostate, left anterior mid
 D22: prostate, left posterior proximal
 D23: prostate, left anterior proximal
 E1,2: right paracaval lymph nodes
 E3,4: lymph nodes, bisected
 F1,2: right common iliac lymph nodes, bisected
 F3: lymph nodes, bisected
 F4: lymph nodes
 G1: left para-aortic lymph nodes
 G2: lymph nodes, bisected
 H1,2: left common iliac lymph nodes, one bisected
 H3: lymph nodes
 I: right lymph node of Cloquet all embedded
 J1,2: right external iliac lymph nodes
 J3: lymph nodes, two bisected
 K1: right obturator / hypogastric lymph nodes
 K2: lymph nodes, largest bisected
 K3: lymph nodes, bisected
 K4-6: lymph nodes (M6 bisected)
 K6: lymph nodes
 L: left lymph node of Cloquet all embedded
 M1-3: left external iliac lymph nodes, bisected
 M4,5: six lymph nodes, bisected
 N: right pre-sciatic lymph nodes all embedded
 O1-3: presacral lymph nodes all embedded
 P: left pre-sciatic lymph nodes all embedded
 Q: aortic bifurcation all embedded
 R: left proximal ureter all embedded
 S: right proximal ureter all embedded

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MICROSCOPIC DESCRIPTION:

A-C: See final diagnosis.

D: Sections of the bladder show an invasive transitional cell carcinoma broadly extending through the muscularis propria and into the perivesical fat (D1-4). The corresponding, subjacent inked radial margin is negative for tumor. No definite lymphovascular space invasion is identified. The adjacent urothelium along the left lateral wall shows mild to moderate urothelial dysplasia. No carcinoma in situ (flat lesion identified).

Sections of the prostate show a unilateral prostatic adenocarcinoma, comprised of variably sized glands infiltrating the fibromuscular stroma. The tumor is predominantly along the left distal prostate (D19-20). No invasion into the capsule or periprostatic soft tissue is identified. A few areas of atypical glands are also seen in the right prostatic lobe (D15, D18). Two K903 stains performed show intermittent staining of basal cells around critical glands, supporting diagnoses of high grade prostatic intraepithelial neoplasia and atypical basal cell hyperplasia. The apical urethral and radial margins as well as bilateral seminal vesicles and vasa deferentia are all free of tumor.

E-S: See final microscopic-diagnosis.

Signed by:

ELECTRONIC SIGNATURE

POST-OPERATIVE DIAGNOSIS:

Bladder Cancer

PRE-OPERATIVE DIAGNOSIS:

Bladder Cancer

ORDERING PHYSICIAN:

Ordering Physician:

PROSECTOR:

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History	☒	
Inc./Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials:	DATE Reviewed: 10/24/2013	